

**For the use only of a Registered Medical Practitioner or a Hospital or a
Laboratory**

For Dermatologic Use Only

Not For Ophthalmic Use

For External Use Only

**MYCLOB 0.05% (CLOBETASOL PROPIONATE SCALP
APPLICATION 0.05% W/W)**

COMPOSITION

Each 1 gm contains:

Clobetasol Propionate Ph. Eur 0.5 mg

PHARMACEUTICAL FORM

Colourless Solution

PHARMACOLOGICAL ACTION

Pharmacotherapeutic group: Corticosteroids, very potent (group IV),

ATC code: D07AD

Mechanism of action

Topical corticosteroids act as anti-inflammatory agents via multiple mechanisms to inhibit late phase allergic reactions including decreasing the density of mast cells, decreasing chemotaxis and activation of eosinophils, decreasing cytokine production by lymphocytes, monocytes, mast cells and eosinophils, and inhibiting the metabolism of arachidonic acid.

Pharmacodynamic effects

Topical corticosteroids have anti-inflammatory, antipruritic, and vasoconstrictive properties.

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Pharmacokinetics:Absorption

Topical corticosteroids can be systemically absorbed from intact healthy skin. The extent of percutaneous absorption of topical corticosteroids is determined by many factors, including the vehicle and the integrity of the epidermal barrier. Occlusion, inflammation and/or other disease processes in the skin may also increase percutaneous absorption.

Distribution

The use of pharmacodynamic endpoints for assessing the systemic exposure of topical corticosteroids is necessary due to the fact that circulating levels are well below the level of detection.

Metabolism

Once absorbed through the skin, topical corticosteroids are handled through pharmacokinetic pathways similar to systemically administered corticosteroids. They are metabolised, primarily in the liver.

Elimination

Topical corticosteroids are excreted by the kidneys. In addition, some corticosteroids and their metabolites are also excreted in the bile.

INDICATION

Psoriasis and recalcitrant eczemas of the scalp. Clobetasol propionate is a highly active topical corticosteroid which is indicated for use in short courses for conditions which do not respond satisfactorily to less active steroids.

CONTRAINDICATIONS

Hypersensitivity to the preparation, infections of the scalp, dermatoses in children under one year of age including dermatitis

DOSAGE AND ADMINISTRATION

Wash and dry affected area. Apply Cloderm Scalp Application sparingly to the affected areas and rub in once or twice daily as directed by your doctor or pharmacist. If symptoms persist

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please consult your doctor or pharmacist. This preparation is for the treatment of your current condition only. Do not allow use by any other person and dispose of unused material when your treatment is complete.

ROUTE OF ADMINISTRATION

Topical Scalp Application

ADVERSE REACTIONS

Infections and Infestations

Very rare Opportunistic infection

Immune System Disorders

Very rare Hypersensitivity, generalised rash

Endocrine Disorders

Very rare Hypothalamic-pituitary adrenal (HPA) axis suppression:

Cushingoid features: (e.g. moon face, central obesity), delayed weight gain/growth retardation in children, osteoporosis, glaucoma, hyperglycaemia/glucosuria, cataract, hypertension, increased weight/obesity, decreased endogenous cortisol levels, alopecia, trichorrhexis

Skin and Subcutaneous Tissue Disorders

Common Pruritus, local skin burning /skin pain

Uncommon Skin atrophy*, striae*, telangiectasias*

Very rare Skin thinning*, skin wrinkling*, skin dryness*, pigmentation changes*, hypertrichosis, exacerbation of underlying symptoms, allergic contact dermatitis/dermatitis, pustular psoriasis, erythema, rash, urticaria, acne

*Skin features secondary to local and/or systemic effects of hypothalamic-pituitary adrenal (HPA) axis suppression.

General Disorders and Administration Site Conditions

Very rare Application site irritation/pain

Eye disorders

Not known: Vision, blurred (see also warning and precautions)

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WARNINGS & PRECAUTIONS

Care must be taken to keep the preparation away from the eyes.

Patients should be advised to avoid:

- smoking whilst applying to the scalp
- fire, flame and heat including use of hair dryer after application

Clobetasol should be used with caution in patients with a history of local hypersensitivity to other corticosteroids or to any of the excipients in the preparation. Local hypersensitivity reactions may resemble symptoms of the condition under treatment.

Manifestations of hypercortisolism (Cushing's syndrome) and reversible hypothalamic-pituitary-adrenal (HPA) axis suppression, leading to glucocorticosteroid insufficiency, can occur in some individuals as a result of increased systemic absorption of topical steroids. If either of the above are observed, withdraw the drug gradually by reducing the frequency of application, or by substituting a less potent corticosteroid. Abrupt withdrawal of treatment may result in glucocorticosteroid insufficiency

Risk factors for increased systemic effects are:

- Potency and formulation of topical steroid
- Duration of exposure
- Application to a large surface area
- Use on occluded areas of skin (e.g. on intertriginous areas or under occlusive dressings)
- Increasing hydration of the stratum corneum
- Use on thin skin areas
- Use on broken skin or other conditions where the skin barrier may be impaired
- In comparison with adults, children and infants may absorb proportionally larger amounts of topical corticosteroids and thus be more susceptible to systemic adverse effects. This is because children have an immature skin barrier and a greater surface area to body weight ratio compared with adults.

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Paediatric population

In infants and children under 12 years of age, long-term continuous topical corticosteroid therapy should be avoided where possible, as adrenal suppression can occur.

Children are more susceptible to develop atrophic changes with the use of topical corticosteroids.

Duration of treatment for children and infants

Courses should be limited if possible to a few days and reviewed weekly.

Infection risk with occlusion

Bacterial infection is encouraged by the warm, moist conditions within skin folds or caused by occlusive dressings. When using occlusive dressings, the skin should be cleansed before a fresh dressing is applied.

Use in Psoriasis

Topical corticosteroids should be used with caution in psoriasis as rebound relapses, development of tolerances, risk of generalised pustular psoriasis and development of local or systemic toxicity due to impaired barrier function of the skin have been reported in some cases. If used in psoriasis careful patient supervision is important.

Visual disturbance

Visual disturbance may be reported with systemic and topical corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes which may include cataract, glaucoma or rare diseases such as central serous chorioretinopathy (CSCR) which have been reported after use of systemic and topical corticosteroids.

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Concomitant infection

Appropriate antimicrobial therapy should be used whenever treating inflammatory lesions which have become infected. Any spread of infection requires withdrawal of topical corticosteroid therapy and administration of appropriate antimicrobial therapy.

INTERACTIONS WITH OTHER MEDICAMENTS

Co-administered drugs that can inhibit CYP3A4 (eg ritonavir and itraconazole) have been shown to inhibit the metabolism of corticosteroids leading to increased systemic exposure. The extent to which this interaction is clinically relevant depends on the dose and route of administration of the corticosteroids and the potency of the CYP3A4 inhibitor.

PREGNANCY AND LACTATION

Pregnancy

There are limited data from the use of clobetasol in pregnant women.

Topical administration of corticosteroids to pregnant animals can cause abnormalities of foetal development

The relevance of this finding to humans has not been established. Administration of clobetasol during pregnancy should only be considered if the expected benefit to the mother outweighs the risk to the foetus. The minimum quantity should be used for the minimum duration.

Breast-feeding

The safe use of topical corticosteroids during lactation has not been established.

It is not known whether the topical administration of corticosteroids could result in sufficient systemic absorption to produce detectable amounts in breast milk. Administration of clobetasol during lactation should only be considered if the expected benefit to the mother outweighs the risk to the infant.

If used during lactation clobetasol should not be applied to the breasts to avoid accidental ingestion by the infant.

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Fertility

There are no data in humans to evaluate the effect of topical corticosteroids on fertility

Clobetasol administered subcutaneously to rats had no effect upon mating performance; however, fertility was decreased at the highest dose

EFFECT ON ABILITY TO DRIVE AND USE MACHINES

There have been no studies to investigate the effect of clobetasol on driving performance or the ability to operate machinery. A detrimental effect on such activities would not be anticipated from the adverse reaction profile of topical clobetasol.

OVERDOSE AND TREATMENT

Symptoms

Topically applied clobetasol may be absorbed in sufficient amounts to produce systemic effects. Acute overdosage is very unlikely to occur, however, in the case of chronic overdosage or misuse the features of hypercortisolism may occur (see side effects).

Management

In the event of overdose, clobetasol should be withdrawn gradually by reducing the frequency of application or by substituting a less potent corticosteroid because of the risk of glucocorticosteroid insufficiency.

Further management should be as clinically indicated or as recommended by the national poisons centre, where available.

STORAGE

Store below 30°C. Do not refrigerate.

PRESENTATION

50mL HDPE rectangle opaque white bottle (20 mm neck) with a 15 mm white coloured LDPE nozzle and white coloured HDPE cap. One bottle is placed in an outer carton along with a pack insert.

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Manufactured in India for :

MACLEODS PHARMACEUTICALS LTD.

by: Oxalis Labs

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Himachal Pradesh- 174 101, India.

Product Registration Holder

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